

Treat the patient according to her current situation:

Starting or changing contraception

Use steps 1-4 to help patient to choose best option according to her needs ↪ 156.

Already using contraceptive

- If patient satisfied with method, check method is still suitable.
- If using IUD or subdermal implant, check when replacement needed.

Recent delivery, miscarriage or termination of pregnancy (TOP)

- Insert IUD within 48 hours of delivery/miscarriage or TOP if no reason to avoid ↪ 157, otherwise can be inserted ≥ 4 weeks or
- Insert subdermal implant, or start injectable or POP at any stage, or
- Offer sterilisation (tubal ligation), if appropriate, or
- Start COC or POP immediately after miscarriage/TOP.
- Start POP immediately after delivery:
 - Avoid COC for 6 weeks after delivery.
 - Avoid COC for 6 months if breastfeeding.

Menopausal

- If < 50 years, give contraception for 2 years after last period.
- If ≥ 50 years, change to progestogen-only or non-hormonal contraceptive until 1 year after last period.

Manage the patient who has missed an injection or pill:

Late injection

- If ≤ 4 weeks late: give the injection.
- If > 4 weeks late: exclude pregnancy ↪ 157.
- If not pregnant, give injection and use condoms/abstain for 7 days. If unprotected sex in past 5 days, offer emergency contraception ↪ 154.

Missed progestogen-only pill (> 3 hours late)

- Take pill as soon as remembered, continue pack.
- If unprotected sex in past 5 days, also offer emergency contraception ↪ 154.

Missed combined oral contraceptive (> 24 hours late)

- If 1 active pill missed: take 1 pill immediately and take next pill at usual time.
- If ≥ 2 active pills missed during:
 - First 7 active pills: offer emergency contraception ↪ 154, and restart active pills 12 hours later.
 - Middle 7 active pills: take the most recent missed pill immediately (discard others). Continue remaining pills as usual. No emergency contraception required.
 - Last 7 active pills: finish active pills of current pack. Omit inactive pills. Immediately start active pills of next pack.

Manage contraception side effects

Side effect	Management
Menstrual abnormalities	Reassure that menstrual abnormalities usually resolve within 3-6 months. If no periods or periods irregular, heavy or painful: assess and manage ↪ 57.
Headaches	• Reassure that headaches usually resolve within 3 months. If headaches persist, consider switch to non-hormonal method like copper IUD. • If using LNG-IUD and first time migraines/severe headaches: refer.
Sexual problems	• If using IUD: if irritation of partner's penis during sex: cut IUD strings shorter. • Reassure that contraceptives unlikely to affect sexual function. If sexual problems persists, consider alternative method: consider IUDs, or subdermal implant or sterilisation if appropriate.
Acne	Reassure that acne usually resolve within 3 months. If problem, consider switch to COC.
Weight gain	Reassure this is often a temporary side effect due to fluid retention, and resolves within 3 months. If BMI > 25, assess CVD risk ↪ 127. Advise healthy lifestyle.
Breast tenderness	Reassure that this usually resolves within 3 months. Advise to wear supportive bra.
Moodiness	Reassure this should resolve. If persists, assess for low mood, stress or anxiety ↪ 86 or consider switch to non-hormonal method like copper IUD.
Red, swollen, painful wound after implant insertion or sterilisation	• Refer to doctor, if available: clean with saline or antiseptic solution (diluted povidone iodine or chlorhexidine solution), remove slough—leave on for 5-15 minutes. Apply an antiseptic ointment (silver or povidone iodine or chlorhexidine or honey) and moisture absorbent dressing. Change dressing daily. • If surrounding tissue involved, give flucloxacillin 500mg 6 hourly or cefalexin 500mg 6 hourly for 5 days. If unwell with fever or pulse > 100, refer.

Review the patient on contraception

- If IUD or subdermal implant inserted this visit, review in 3-6 weeks. Thereafter, review yearly as needed.
- If oral or injectable contraceptive started this visit, review in 3 months. Thereafter, review as needed.